The Apprenticeship of Philippe Pinel: A New Document, "Observations of Citizen Pussin on the Insane"

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Philippe Pinel is usually pictured in histories of psychiatry and of medicine as the man who first liberated the insane from their chains. The document presented here, discovered in 1978 in the Archives nationales in Paris, reveals the crucial role of Jean-Baptiste Pussin, the "governor" of mental patients at Bicêtre. It was Pussin who initiated Pinel to psychological methods (the famous traitement moral*) in dealing with hospitalized mental patients. Further, in 1797, Pussin first replaced chains with straitjackets. Pinel later obtained Pussin's transfer to the Salpêtrière to help him reorganize that huge hospital.*

IN HIS FAMOUS *Treatise on Insanity* of 1801 (1) and repeatedly thereafter, Philippe Pinel (1745–1826) acknowledged his indebtedness to Jean-Baptiste Pussin. Little is known about this unlearned but experienced "governor of the insane" at the Hospice de Bicêtre in Paris. Born in Lons-le-Saulnier in 1746 and by profession a tanner, Pussin was admitted to Bicêtre in 1771 with scrofula and was "cured." As often happened with former patients, he found employment at the hospital, first on the boys' ward and then, in 1784, as superintendent of the ward for incurable mental patients, the seventh ward or *Emploi St. Prix*. That was where Pinel met Pussin in September 1793, at the height of the Reign of Terror. Pinel began his public career at the age of 48 and served for 2 years as Physician of the Infirmaries at Bicêtre, only to be promoted to Physician-in-Chief at the women's hospice, the Salpêtrière. There Pinel lived, taught, wrote the *Philosophic Nosography* (2), the *Treatise on Insantiy* (1), and *Clinical Medicine* (3), and achieved a well-deserved reputation as the finest clinician and alienist in France.

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BACKGROUND

Less well-deserved is Pinel's universal reputation as liberator of the insane, and the document presented here, long lost in the Archives nationales of Paris until its discovery in the summer of 1978, should lay the myth to rest. Written by Pussin himself, apparently in response to a request from Pinel, it gives us Pussin's own account of his treatment of the insane at Bicêtre and for the first time adequately documents his central role in the reform of patient care, traditionally attributed to Pinel.

It is true that the myth of Pinel the liberator is enshrined in every history of psychiatry and most histories of medicine and is fixed in two well-known paintings. One by Charles Müller, dated 1849 and enthroned in the entrance hall of the Academy of Medicine in Paris, is entitled "Pinel Orders the Chains Removed from the Insane at Bicêtre"; the other, even better known, by Tony Robert-Fleury and painted as late as 1878, is called "Pinel Frees the Madwomen at the Salpêtrière," where it is displayed. The years 1849 and 1878 are of course dates that mark the Second and Third French Republics. The political significance that underscores the myth of Pinel the "liberator" was reenforced when his statue at the Salpêtrière was unveiled on July 13, 1885. In their fight against catholicism and monarchism, republicans claimed Pinel for science and democracy.

That Pinel was not the first or the only alienist to advocate kindly treatment of mental patients and replacement of those heavy, degrading iron shackles with straitjackets has long been acknowledged by historians and Pinel himself. His "enlightened" contemporaries in many countries pursued similar aims. Willis, Haslam, and the Tukes in England, Fowler in Scotland, Daquin at Chambéry, Chiarugi at Florence, the Brothers of Charity, and the French military medical inspector Jean Colombier used and advocated what Pinel would call "psychological treatment" (*méthode morale*) in caring for the mentally ill.

But now, in this newly discovered manuscript from Pussin's own hand, we learn that even at Bicêtre it was Pussin and not Pinel who removed the chains and that psychological treatment of the insane was the rule long before Pinel's arrival. The report of the Poverty Committee of the National Assembly, dated 1790, had already recorded this humane attitude. Now we read that Pussin forbade cruelty toward patients and rou-

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tinely dismissed insubordinate attendants. Pinel, so justly famous for conceiving of mental illness as *illness* within a medical framework, obviously recognized in Pussin an expert. And thus this learned middle-aged physician virtually apprenticed himself to the untutored but experienced governor of the insane, went on daily rounds with him, observed, asked questions, and took notes.[1]

Transferred to the Salpêtrière in 1795, Pinel wished for Pussin's presence in this huge hospice that housed about 6,000 women. The present document suggests that in order to obtain Pussin's transfer, Pinel asked the superintendent to record his therapeutic and administrative strategies. The document's eight headings appear to be answers to the following questions that Pinel had asked:

1. What is the number of insane patients admitted? Has the Revolution brought changes?
2. What is the incidence of mortality?
3. What therapy succeeds best?
4. Is work useful to patients?
5. Is bleeding helpful?
6. What diet should the insane receive?
7. Does the treatment administered at the Hospice of Humanity [the Hôtel-Dieu de Paris] aid recovery?
8. What methods work best in dealing with insane patients?

Pussin added tables that showed the number of patients admitted, "dismissed after cure," dead, and remaining year by year, from 1784 to September 1797. The tables classified the patients by ages; they indicated a mortality of well over 50% before the Revolution, with 95 of the 151 insane dead at Bicêtre in 1788, the terribly cold and barren year that led to the great upheaval of 1789. The highest mortality usually occurred among men between the ages of 20 and 40; the death rate fell drastically during the last 2 years it was tabulated, and we are left to wonder why. Equally intriguing is the category "dismissed after cure"; we must trust Pussin and Pinel to have recognized mental health when they saw it.[2]

Pussin's "Observations" served Pinel's purpose well. The physician-in-chief sent them to the Minister of Internal Affairs, François de Neufchateau, with a long covering letter dated October 17, 1798, in which he justified his request for Pussin's transfer to the Salpêtrière (see figure 1).

> I have become convinced that strict regulations for mental patients are invaluable [Pinel argued]. The supervisor at Bicêtre, citizen Pussin, has introduced and consistently followed such rules. I now strongly wish to see the same principles applied to the women patients at the Salpêtrière.
>
> Without an established method in the management of these patients, confusion and disorder reign. I wish here to testify to my personal appreciation for citizen Pussin. He combines a rare intelligence and several years' experience with humane feelings and an unshakeable firmness. This is particularly important to handle the servants and to dominate the minds of certain mental patients; it greatly helps to reestablish their reason. I have now worked at the Salpêtrière for three years. I have been unable to undertake the therapy of madness or even to make any exact observation on this illness because of the disorganization prevalent in that part of the hospice. . . .

The transfer was approved, perhaps in part because of its endorsement by the Ideologue Georges Cabanis, then a member of the Council of Five Hundred, and by Thouret, the Dean of the Paris Medical School. Both men were Pinel's friends. Pussin finally arrived at the Salpêtrière on May 19, 1802, and spent the last 10 years of his life as Pinel's collaborator. He died in 1811.

Pussin's manuscript is preserved among François de Neufchateau's papers in two parts. Part I consists of the tables for the period 1784–1792, with Observations that are conventional and unenlightening. It may be that Part I was composed at Pinel's request, for it is dated September 22, 1793, 11 days after Pinel's arrival at Bicêtre as Physician of the Infirmaries.

Part II is unquestionably that long-lost "manuscript of nine pages in my possession" to which Pinel referred in the Introduction to the first edition of the *Traité médico-philosophique sur l'aliénation mentale ou la manie* (1). The tables complement and complete the earlier ones; together they cover the period January 1784–September 1797. Pussin's Observations in Part II are outstanding for their modernity, humanity, and self-assurance. He emphasized the importance of firm rules and kindly treatment, the futility and danger of bleeding, the therapeutic value of work, the aptness of former patients as psychiatric nurses, and the need for separate, specialized mental hospitals.

Several of Pussin's remarks and episodes were later repeated by Pinel. In fact, an apposition of some of Pinel's later writings with these nine pages gives striking proof of Pinel's indebtedness to Pussin. On the other hand, a comparison of Pussin's Observations of 1797 with those of 1793 shows a changed man: open-minded, informative, observant, self-reliant in his use—but not abuse—of power, confident with regard to therapy, proud of the recoveries he facilitated. Pussin did not mention his famous associate, nor did he give credit to his own wife who, according to Pinel, was his collaborator and displayed crucial acumen and courage in handling violent patients.

The present document does not detract from Pinel's greatness. It shows us a medical theorist who welcomed the practical wisdom of a humble layman and adopted his empirical evidence. It also proves that the pivotal change in attitude toward mental patients sprang from the "spirit of the times" in the Enlightenment, motivating outstanding physicians like Pinel as

[1]Readers should be alerted to the fact that the only available English translation of Pinel's Introduction to his *Treatise on Insanity*, in which he describes this experience, is in Zilboorg's *History of Medical Psychology* (4). It is so garbled as to be incomprehensible (see especially p. 340).

[2]These tables and the French text of Pussin's Observations can be found in "The Apprenticeship of Philippe Pinel" (5).

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FIGURE 1
Letter from Philippe Pinel to the Minister of Internal Affairs, François de Neufchâteau, Dated October 17, 1798, Requesting Jean-Baptiste Pussin's Transfer from Bicêtre to the Salpêtrière[a]

pendant le temps que j'ai pris part à
l'administration des hospices civils
de Paris, j'ai reconnu le Cit.
Pussin pour un homme honnête, ferme et capable. tout
Cependant de lui le Cit.
Pinel est la seule vérité.

Cabanis
membre de l'Inst.

pour avoir fait les observations
du C. Pussin je suis convaincu
qu'il est un des hommes les plus
utiles pour la classe des aliénés
dont il s'est occupé. Rousselmédecin

au Citoyen Ministre
de l'intérieur

Citoyen Ministre

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Les fonctions de médecin en chef que j'ai remplies successivement dans l'hospice de Bicêtre et celui de la Salpêtrière, m'ont convaincu des avantages précieux de la police intérieure introduite et strictement observée parmi les insensés de Bicêtre par le citoyen Pussin qui en est le surveillant; et puis je ne puis désirer vivement de voir les aliénées de la Salpêtrière dirigées suivant les mêmes principes?

tout devient confusion et désordre sans une méthode invariable dans la direction de cette classe d'infirmes, et je dois ici rendre un témoignage authentique au citoyen Pussin qui joint à une intelligence rare et à une expérience de plusieurs années, l'heureux

[a]The letter was favorably endorsed by Georges Cabanis, Mourgue, Roussel, and the Dean of the Medical School, Michel-Augustin Thouret (Thouret's recommendation does not appear here).

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FIGURE 1 (continued)

accord des sentimens d'humanité et d'une fermeté
imperturbable, si necessaires pour contenir les gens
de service, prendre de l'ascendant sur l'esprit des
certains aliénés et concourir ainsi puissamment à
retablir leur raison. Depuis plus de trois ans que
je suis en activité de service à la Salpetriere il
m'a été impossible d'entreprendre le traitement de
la folie ou même de faire aucune observation exacte
sur cette maladie, à cause de l'espèce de desorganization
où est cette partie de l'hospice.

Je demande donc que le Citoyen Pussin soit
dorénavant chargé de la Police interieure des
aliénées de la Salpetriere, et je vous prie, Citoyen
Ministre, de le recommander à la sollicitude de
l'administration des hospices civils afin qu'elle
statue sur son deplacement et qu'elle prenne toutes
les mesures que sa prudence et la connoissance des
localités pourront lui suggerer. Salut et Respect

Maison Nationale de la Salpetriere
ce 26e Vendemiaire an 7e

Pinel médecin en chef et Professeur à l'école de médecine

well as uneducated but experienced and humane individuals like Jean-Baptiste Pussin.[3]

[3]For a fuller treatment of Pinel's formative years, the reader may wish to refer to my work "Health and Mental Health in the Thought of Philippe Pinel: The Emergence of Psychiatry during the French Revolution" (6). This material was first presented as a special lecture at the 129th annual meeting of the American Psychiatric Association in 1976.

"OBSERVATIONS OF CITIZEN PUSSIN ON THE INSANE"

1. Since 1791 (old style) inclusively, only half as many madmen as before have entered my ward every year. Why? Were there fewer than before? Certainly not, by any means. But only those presumably incurable were sent to me. The others, provided they seemed calm, and on the mere indication of the male servants

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at the Hospice of Humanity, were sent directly from that hospice to the sixth ward. What could be the reason for this regulation?

2. During the food shortage, and because of it, mortality was considerable and claimed many inmates whose cure was progressing well. The only ones who escaped and reentered society were those whose parents came to their help by bringing them some food or those whom I could employ as servants, which provided similar aid for them.

3. Moderate work and distraction are very favorable to the recovery of these unfortunates. I have often noticed that when I employed a madman who had just recovered his senses either to sweep or to assist a servant, and then to become himself a servant—I have noticed, I say, that his state improved every month, and that somewhat later he was totally cured. There are few instances where this method has not succeeded. Therefore, as much as possible, all servants are chosen from the category of mental patients. They are at any rate better suited to this demanding work because they are usually more gentle, honest, and humane.

4. Work also seems to me the only means, if not of cure, at least of relief for those whose madness is caused by pride or by religion, and who have hitherto been considered as almost totally incurable. I have not been able to test this, for fear of evasion, since they are always very dangerous to society. It is true that they seek isolation, the former because they usually despise everybody, the latter because, thinking they are God or his messengers, consider all men as beneath them and are convinced that they dispose of the right of life and death over everything that exists on earth. There is one patient in particular on this ward who pretends to be Christ; he began by killing his two children at home and was thereupon imprisoned at Nancy and placed in a cell with a companion whom he killed. Brought finally to Bicêtre, he would have slit the throat of a madman and killed him like the others, had I not surprised and stopped him and thus saved the victim. Punishment? Fear? Threats? Nothing can change his feelings. He confronts everything, even death, being convinced that it would be the greatest honor for him thus to imitate his master, by a final martyrdom. . . . Fortunately there are few of this kind.

I say that, since there is hardly any hope of cure after a number of years, the greatest service one could render these men is to give them work to do. Almost all of them are able to work and would do so gladly, if one could give them a little encouragement. Also their work would help to cover a part of the cost of their upkeep and somewhat lessen their unhappiness.

5. I have noticed that excessive bleeding is dangerous for most madmen because it causes the illness to degenerate into imbecility. Inmates often arrive from the Hospice of Humanity whom bleeding has reduced to such a state that they are even unable to eat. And yet, when they are young and under thirty years old, it is rare that the strength of their temperament does not bring about a favorable change. This state of imbecility is not limited in time, but ordinarily lasts from two months to one year, more or less. When an improvement is about to occur, one perceives a slow and gradual effervescence in the individual's blood. The fever follows the same course. At the crisis point the imbecile has turned into a furious madman. This salutary crisis sometimes lasts one or several months, but in the end it guarantees a perfect cure; and what is extraordinary is that the fever, or rather the crisis, often stops suddenly. That is the moment, especially in winter, where the closest supervision must be provided because during this bout they cannot tolerate anything to cover them and are consequently all naked and would catch cold and die if they were not promptly helped.

I could cite many examples, but shall limit myself to one. A certain Louis B . . ., sixteen years old, the son of a blacksmith from La Villette, was treated at the Hospice of Humanity. He was bled, and the ligature loosened during the night; he lost blood and was reduced to such a state of weakness that he remained unconscious for several hours, despite all attempts to help him. He was later brought to Bicêtre and, to give an idea of his condition, I shall say that he was too weak to raise himself, with a fixed stare like a newborn infant and he had to be cared for like one, being unable to do anything by himself. He remained in this state for about two months. Then the crisis happened and lasted one month, after which he was perfectly cured. At the beginning of this crisis his father came to visit and lamented his fate: this was his only boy, who would surely never recover his health, seeing his sad condition. But since I perceived the beginning of a crisis, I assured the father that, within one month, there would be a considerable change in his son whose early recovery I predicted. Nothing I said convinced him, but one can imagine his surprise when, after the appointed time, he found his son entirely cured. Above the age of thirty, this kind of cure is rare, and I know only Citizen D . . ., now in Paris, to have been cured of imbecility at the age of about forty.

6. I presume that food which keeps the belly free would be best for all madmen, particularly for those suffering from periodic, especially annual, madness. I have been observing these illnesses for several years and searching for the cause of their return. I have totally convinced myself that the attack is always preceded by constipation. I have seen patients, even lately, who have not been to the toilet for five days. The effect is quite obvious since their heads and upper bodies are very inflamed, and usually the attack ends only with a bowel movement. For those with annual madness and the most violent ones, I have repeatedly used the only means at my disposal: it is to administer a decoction of wild chicory as soon as I notice the beginning of an attack. The use of this infusion for two weeks has repeatedly been sufficient to arrest and neutralize the attack and thus rid them of the illness for that year. Thus I am sure that, if this treatment were

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followed, they could ultimately return into society. It is their cruel fate always to be incarcerated without hope of freedom, because of an illness that takes away their judgment for only six weeks or two months and leaves them aware of their sad state all the rest of the year.

7. As soon as a madman has been treated at the Hospice of Humanity, he is rightaway transferred to Bicêtre. There one can judge whether the therapy was favorable or harmful. Some are so weak that they die during the first month; others remain imbecile. This regrettable situation will last as long as there is no special hospice where one can begin therapy and pursue it until recovery is complete. It is indeed clear that even the health officers' observations are always faulty and therefore almost useless because, once the patient has left the hospital, there is no way of knowing whether the treatment has produced the expected effect.

8. Until now, and in most hospices, furious madmen have always been considered and treated like wild beasts. How long will such an insane and barbarous treatment be followed? How many hapless heads of family have been its victims! When it is so easy to find means of repression without harming people, as I shall tell shortly. Without any doubt, mistreatment angers the insane rather than calming them. Beatings may well tame them for the moment, but they will assuredly not forget. The memory of mistreatment leads the inmates to watch for the moment of vengeance as soon as the opportunity arises and when one expects it the least. What is more, while they are consumed by this thirst for vengeance, anything one might undertake for their cure is, if not harmful, at least absolutely useless. But, as I have already stated, one can use means of repression without mistreatment and even with appropriate tact. One can thereby avoid irritating them and thus induce them, in their lucid moments, to regard you as their benefactor and not as their enemy. I have the most convincing proofs for everything I am saying here.

When, thirteen years ago, I was made supervisor of the insane at Bicêtre, I saw only men full of hatred and vengeance against all the administrative personnel around them. I almost fell victim to it myself when a priest, Citizen A . . ., a strong and powerful man, grabbed me by the hair from behind and hit me several times on the head with the back of his breviary—so hard that I lost consciousness on the spot and, without the attendants who plucked me from his hands, I would unquestionably have perished.

I held to the principle that in no case would I permit the insane to be beaten and I had formally declared my intentions in this respect. The attendants tried to rebel against me, saying that they were not safe and objecting that, if I myself was not spared, they were all the more exposed, etc. But, despite their clamors, I persisted in my resolve and, to reach my goal, I was forced to dismiss almost all of them in turn when they disobeyed. It is thus not without trouble that I realized my purpose, but I finally managed to achieve that the servants never beat any of the insane, even when victimized by their violence. I know that those who care for the insane run great risks, but I am also certain that the danger is less when gentleness rather than severity prevails.

I have had as many as three hundred madmen under my supervision and many of them have always been violent and very dangerous, especially during the hot season. To control them I have never used anything but repressive measures without mistreatment, and I have never permitted that they be beaten in any way. And yet I have always managed not only to impress them but even to gain their confidence to such a point that they are the first to protect me. They even help maintain order and calm in their own midst and these regulations are so well observed that most of the time one does not seem surrounded by madmen. Strangers have often told me of their surprise. It is true that very active supervision is required to achieve this.

I have tried so hard to improve the condition of these unfortunates that in the month of Prairial of the Year V I managed to eliminate their chains (used until then to contain the furious) and to replace them with straitjackets that permit freedom of movement and the enjoyment of all possible liberty without any added danger.

This class of men which has until now been virtually abandoned is nevertheless most worthy of attention since their sensitivity is the cause of their illness. If one seeks upright men, it is among these that a large number can be found. And, in any case, who can flatter himself to have spent his life without any misfortune and without fear of a similar fate? What government can provide protection against it?

My experience has shown, and shows daily, that to further the cure of these unfortunates one must treat them with as much kindness as possible, dominate them without mistreatment, gain their confidence, fight the cause of their illness and make them envision a happier future. I have always fought this illness by psychological means and thus known the happiness of some favorable results. Yet I am far from denying the usefulness of physical treatments. On the contrary, I believe them to be very advantageous, even urgent, when they are adequately supervised.

Work, among other things, seems to me almost necessary, not only because it provides for exercise but also because it offers distraction. Work, in fact, belongs to the category of psychological remedies on which I especially insist.

It is thus doubtless out of ignorance or by mistake that the Lycée des Arts has, in one of its public sessions, presented the Hospice of Avignon, used for mental patients, as the only one in France where this [psychological] therapy is practiced, whereas I have had no others at my disposal for the past thirteen years.

However that may be, these are the principles that I have always followed in applying the means at my disposal ever since I have been in charge of this ward. I

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say *the means at my disposal* and one knows how limited they are with regard to space, the lack of baths, etc., etc. . . . without mentioning the difficulties and innumerable obstacles constantly encountered. Yet, despite these hurdles, not a year has passed when I have not returned several madmen to society. . . . And how much larger this number would have been with the resources that an establishment of this kind requires.

Done at Bicêtre, the 1st Nivôse, Year VI of the French Republic [December 21, 1797]

Pussin

Supervisor of the insane in the said establishment for almost 28 years

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